

The Royal Hospital Handover Policy

1. Introduction:

Handover between healthcare providers plays a crucial role in maintaining continuity of care and ensuring patient safety. Communication failures are known to occur during the handover of patients from one healthcare provider to another. Common communication failures during handovers include incomplete or inaccurate transfer of essential patient information, failure to address important patient concerns or issues, miscommunication about medication orders or treatments, and inadequate communication regarding patient allergies or medical history. These failures can lead to medical errors, compromised patient safety, and disruption of continuity of care.

2. Scope:

The Royal Hospital's clinical handover policy and procedures are meant to be followed and implemented by all healthcare professionals working in all clinical areas in all hospital departments.

3. Conditions:

- 3.1 In all in-patient settings, patient care is transferred between the medical and on-call teams.
- 3.2 patients' care is transferred between healthcare professionals during shift changes.
- 3.3 During patients' transfer from one department to the other.
- 3.4 During the transfer of the Royal Hospital patients to other healthcare facilities

4. Aim:

- 4.1 To establish and implement the best practices for effective handover communication using standardized handover tools, protocols, and huddle areas. This will facilitate the secure and accurate transfer of patient information between different healthcare professionals.
- 4.2 To ensure proper continuity of patient care and completion of any pending work by the incoming on-call team using a shared mental model between ongoing and incoming teams.
- 4.3 To introduce a handover monitoring plan at the departmental level and to be directly communicated to the hospital quality department.

5. Handover tools used in the Royal Hospital

- 5.1 **IPASS handover tool** used in various hospital departments
- 5.2 **Richman handover tool** used in the NICU
- 5.3 **ER handover** used in ER department
- 5.4 **SBAR tool** used by the Nursing department

6. Definitions:

- 6.1 **Clinical handover:** refers to the transfer of professional responsibility and accountability for some or all aspects of care for a patient, or group of patients, to another person or professional group on a temporary or permanent basis.¹

6.2 Characteristics of high-quality handover:

- The handover leader designates roles for team members.
- Clear transfer of information, tasks, and responsibilities is essential.
- A dedicated time and space for the handover must be established.
- Information should be communicated verbally in person and supported with written documentation.
- A standardized format should be used to ensure that the information is current, accurate, and relevant.
- The handover should follow a structured process, starting with a high-level overview.

- Employ a closed-loop communication system to enhance situational awareness and establish a shared mental model.

6.3 Situational awareness: is a cognitive skill that involves knowing what is going on around you. It involves managing individual issues while remaining focused on the "big picture." Medical decision-making relies heavily on situational awareness, which is one of the most important non-technical skills.²

6.4 Team situational awareness: Refers to the shared understanding of an evolving situation among team members. It involves knowing the tasks and responsibilities of members of a team. Team situation awareness is achieved when team members share their knowledge and analysis of a situation with other team members.³

7. Roles and responsibilities:

Directors/Head of the department: Designate “a handover team”, which is a group of doctors who will facilitate and coordinate the handover process for their department. This team will serve as the point of contact for their department with the handover project team and the quality department. Additionally, by receiving training, this team will be able to educate and guide their department members on best practices for the handover process and monitor adherence.

Unit head: Ensures the effective implementation of the handover procedure using the recommended handover tool by his/her respective unit in collaboration with the handover team in their department.

All doctors (MDs): Implement and adhere to the handover policy and its processes supported by guidance from their departmental handover team.

Quality Department: monitors handover compliance and receives quarterly reports of handover audits from hospital departments.

8. Procedure:

- 8.1 All patients’ care should be safely transferred between healthcare professionals at the time of a change in duty hours.
- 8.2 All physicians working at any department at the Royal Hospital should use the IPASS handover tool as their standardized handover tool in daily bases. (Details of this handover tool is in Appendix 1).
- 8.3 The handover process should include verbal & written information about all the patients.
- 8.4 Each department assigns a specific suitable handover timing and adheres strictly to the agreed timing.
- 8.5 In the Emergency Departments, Handover should take place at the time of shift changes (3 times/day)
- 8.6 Each department should monitor adherence to the handover policy and standards using the audit tool provided in Appendix 2. Other audit tools could be used to monitor compliance.
- 8.7 A report about the handover implementation and adherence should be submitted quarterly by the handover team of each department (every 3 months) directly to the Quality and Patient Safety Department.

Note: Refer back to the following policies for further information

- **Nursing clinical handover SBAR**
- **Patients’ information transition during doctors’ rounds and nursing handover neonatal SBAR (RICHMEN)**

- **Handover policy in ED**

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Version	1.0
Reviewed by	
Approved by	
Date Approved	December 3 rd 2024
Due review date	January 25 th 2027

Appendix 1 : The I-PASS Handover tool with an example



The I-PASS Mnemonic

I	Illness Severity	Stable, “watcher,” unstable
P	Patient Summary	- Summary statement - Events leading up to admission - Hospital course - Ongoing assessment - Plan
A	Action List	- To do list - Timeline and ownership
S	Situation Awareness and Contingency Planning	- Know what’s going on - Plan for what might happen
S	Synthesis by Receiver	- Receiver summarizes what was heard - Asks questions - Restates key action/to do items

Starmer. Pediatrics. 2012 Feb;129(2):201-4.

Illness severity	Patient Summary	Action list Situational awareness
Watcher Name: N.S Age: 6 months Diagnosis: Heart Failure Admit Date: Weight: 6 kg Allergies: no allergies CODE: full code Location: P3 bed 34 Services: Cardiology, HD	HPI: 2 days history of increased work of breathing and poor feeding following URTI symptoms. Admitted for management of heart failure with IV anti-failure medications. Relevant PMHx: VSD Active issues: 1- tachypneic & tachycardiac (frusemide dose adjusted today) 2- Tachypneic and she is on O2 supplement. IVF/TFI: TFR 130ml/kg/day , no IVF. Diet: regular formula = TFR 130ml/kg/day through NGT O2: 1 L of O2 via NC Meds: Frusemide, spironolactone, captopril.	Daytime action list: monitoring respiratory status. Pending <u>ix</u>/consults: Night action list: In/Out , f/u her fluid balance at 20:00 and consider an extra dose of Lasix (3mg) if she is > 200cc positive monitoring respiratory status. Situational awareness: Have a low threshold to call HD/PICU.

Appendix 2 : Handover Audit tool

I-PASS Printed Handover Document Assessment

Date and time this tool printed: _ _ / _ _ / _ _ (dd/mm/yy) time: _ _ : _ _ AM / PM Service: _____

1. How well do you know the patients on the printed handover document? ☐ Very well ☐ Somewhat well ☐ Not at all

2. Number of patients on printed handoff document: _____

Indicate how frequently each element of the I-PASS mnemonic is present on the printed handoff document.

Mnemonic	Description	Never	Rarely	Sometimes	Usually	Always
3.Illness Severity	Identification as stable, “watcher”, or unstable					
4.Patient Summary	Summary statement, events leading up to admission, hospital course, ongoing assessment, plan					
5.Action List	To do list; timeline and ownership					
6. Situation Awareness/Contingency Planning	Know what’s going on; plan for what might happen					
7.Synthesis by Receiver	Written reminder to prompt receiver to summarize what was heard during verbal handoff					

8. How often are the following essential elements present and accurate on the printed handover document: • Name • Room # • Weight • Age • Service / Team • Allergies. • Medication name	Never	Rarely	Sometimes	Usually	Always

Rate the frequency with which the printed tool had:	Never	Rarely	Sometimes	Usually	Always
9. Patient summary with clearly specified plan for remainder of admission					
10. To-do items with clear if/then format when appropriate					
11. To-do list restricted to items that should be accomplished on next shift					
12. High quality contingency plans documented for items not on to-do list					

13. Rate the length of the printed handover document:

☐ Very excessive length ☐ Excessive length ☐ Appropriate length ☐ Abbreviated length ☐ Very abbreviated length

Rate the following:	Poor	Fair	Good	Very good	Excellent
14. Accuracy of Illness Severity Assessments					
15. Quality of Patient Summaries					

Rate the frequency with which the printed tool contained the following:	Never	Rarely	Occasionally	Fairly often	Very often
16. Omissions of important information					
17. Irrelevant information					

18. Did you observe any erroneous information on the printed tool? ☐ Yes , if yes, how many times? _____ ☐ No

19. the giver & receiver Self-Reflection	Your Feedback on Self-Reflection	Feedback highlights	Next step
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References

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